



EVALUATION 3 — VERY HIGH ACUITY

Primary Diagnosis: New-Onset Stroke (Left MCA CVA) with Right Hemiparesis, Dysphagia, and Aphasia

PATIENT DEMOGRAPHICS

Field Detail

Patient Name Yuki Tanaka-Morrison

Date of Birth 08/09/1948

Age 76

Sex Female

Race/Ethnicity Asian / Japanese American

Primary Language English (fluent); Japanese (native — reverts to Japanese when fatigued or stressed per family)

Interpreter Needed Yes — Japanese interpreter recommended for complex medical discussions and when patient is fatigued

Marital Status Married

Medical Record Number CI-2025-00431

Address of Service 560 Cherry Blossom Lane, Foster City, CA 94404

Home Phone (650) 555-0388

Cell Phone N/A (patient unable to use; husband cell below)

Email N/A

County San Mateo

EMERGENCY CONTACTS

Priority Name Relationship Phone Lives with Patient?

1 David Morrison Husband (650) 555-0389 (home) / (650) 555-0156 (cell) Yes

2 Keiko Tanaka-Morrison Daughter (415) 555-0733 No — SF (visiting daily through 07/15)

3 Dr. Thomas Morrison Son (physician — internal medicine, lives out of state) (312) 555-0921 No — Chicago, IL

LIVING SITUATION

Field Detail

Lives With Husband David (age 79 — mild arthritis, uses cane, hearing aids; active and cognitively intact; devoted but physically limited)

Residence Type Two-story home, owned (bedroom upstairs; temporary bedroom setup in ground-floor study)

Stairs to Enter None (ground-level entry at garage)

Interior Stairs 14 stairs to second floor (not currently being used — patient confined to ground floor)

Pets None

DME in Home Hospital bed (ground-floor study), Hoyer lift (rented), wheelchair (standard — needs assessment for tilt-in-space), bedside commode, suction machine (portable), pulse oximeter, transfer belt

PAYER INFORMATION

Priority Payer ID / Policy Number Group Number

Primary Medicare Part A (Traditional) 7HK2-AB9-QW15 N/A

Secondary AARP/UnitedHealthcare Medicare Supplement (Plan G) UHC-223456781 N/A

PHYSICIAN INFORMATION

Role Name Specialty Phone Fax

Attending / Certifying Dr. Sarah Lin Neurology (Stroke) (650) 555-1310 (650) 555-1311

PCP Dr. Robert Huang Internal Medicine (650) 555-0580 (650) 555-0581

Physiatrist Dr. Karen Okonkwo PM&R (650) 555-1442 (650) 555-1443

F2F Encounter Date: 06/10/2025 (inpatient — acute stroke) F2F Performed By: Dr. Sarah Lin

REFERRAL / HOSPITALIZATION HISTORY

Event Facility Dates Reason

Acute Hospitalization Stanford Medical Center 05/28/2025 – 06/03/2025 Left MCA ischemic stroke; tPA administered; ICU x 3 days

Inpatient Rehabilitation Peninsula Rehabilitation Center 06/03/2025 – 06/16/2025 Intensive stroke rehabilitation (13 days)

Start of Care Date: 06/17/2025 Referral Source: Peninsula Rehabilitation Center — Dr. Okonkwo / Discharge Planner: Angela Rodriguez, MSW

DIAGNOSES

Priority ICD-10 Code Diagnosis

Primary I63.512 Cerebral infarction due to unspecified occlusion or stenosis of left middle cerebral artery

Secondary I69.351 Hemiplegia and hemiparesis following cerebral infarction affecting right dominant side

Secondary I69.321 Dysphasia following cerebral infarction (expressive aphasia, moderate)

Secondary I69.391 Dysphagia following cerebral infarction

Secondary R13.19 Other dysphagia (modified diet: mechanical soft with thin liquids per SLP assessment at rehab discharge)

Secondary I48.0 Paroxysmal atrial fibrillation (new diagnosis — etiology of stroke; detected inpatient)

Secondary I10 Essential hypertension

Secondary E78.00 Pure hypercholesterolemia, unspecified

Secondary M79.3 Panniculitis, unspecified (chronic right shoulder pain — premorbid)

Secondary N39.0 Urinary tract infection, site not specified (treated inpatient, resolved; Foley discontinued 06/10/2025)

Secondary R39.15 Urgency of urination (new since stroke — neurogenic bladder suspected)

Secondary F06.30 Mood disorder due to known physiological condition, unspecified (post-stroke depression/emotional lability)

Surgical History

Cesarean section x2 (1978, 1981)

Laparoscopic cholecystectomy (2009)

Right carpal tunnel release (2015)

ALLERGIES

Allergen Reaction Severity

Morphine Respiratory depression, severe sedation SEVERE

Metoclopramide (Reglan) Dystonic reaction (tongue protrusion, neck stiffness) SEVERE

Peanuts Anaphylaxis SEVERE

Latex Urticaria Moderate

CURRENT MEDICATIONS

Medication Dose Route Frequency Purpose High-Risk?

1 Apixaban (Eliquis) 5 mg PO (crush and mix with applesauce per SLP) BID Anticoagulation (A-fib — stroke prevention) YES
2 Atorvastatin (Lipitor) 80 mg PO (crush) Daily HS Hyperlipidemia / secondary stroke prevention No
3 Amlodipine 10 mg PO (crush) Daily AM HTN No
4 Lisinopril 20 mg PO (crush) Daily AM HTN No
5 Metoprolol succinate (Toprol XL) 50 mg PO (do NOT crush — extended release) Daily AM Heart rate control (A-fib) Yes
6 Sertraline (Zoloft) 50 mg PO (crush) Daily AM Post-stroke depression No
7 Baclofen 10 mg PO (crush) TID Spasticity (right upper extremity) No
8 Oxybutynin (Ditropan) 5 mg PO BID Urinary urgency / neurogenic bladder No
9 Bisacodyl (Dulcolax) 10 mg PO/PR PRN (if no BM x 3 days) Constipation No
10 Acetaminophen 650 mg PO (crush in applesauce) Q6H PRN Pain (shoulder, general) No
11 Multivitamin 1 tab PO (crush) Daily Nutritional support No
Critical Medication Note: Most PO medications must be crushed and administered with applesauce or pudding due to dysphagia. Exception: Metoprolol succinate (Toprol XL) is EXTENDED RELEASE — must be swallowed whole. If patient cannot swallow whole, notify physician for alternative formulation (metoprolol tartrate — immediate release, crushable, but requires BID-TID dosing).

Controlled Substances in Home: None Sharps in Home: None

CLINICAL ASSESSMENT — VITAL SIGNS AT ADMISSION

Parameter Value Abnormal?
BP 148/82 mmHg (left arm — right arm not used for BP per stroke protocol) Mildly elevated
HR 72 bpm, irregular (intermittent) Irregular — known paroxysmal A-fib

RR 16 WNL

Temp 98.4°F WNL
SpO2 97% on RA WNL
Weight 138 lbs (62.6 kg) — down from pre-stroke 152 lbs Weight loss 14 lbs in 3 weeks (9.2%) — significant
Height 5'3" (160 cm)

BMI 24.4 WNL

Pain 5/10 right shoulder (spasticity-related); 2/10 generalized Yes

CLINICAL ASSESSMENT — NEUROLOGICAL / STROKE-SPECIFIC

Motor

Area Finding
Right Upper Extremity Severe hemiparesis. Shoulder: 2/5 (gravity-eliminated movement only). Elbow: 2/5. Wrist: 1/5 (trace). Hand/grip: 1/5 (trace — unable to grasp objects). Fine motor: absent. Increased tone/spasticity (Modified Ashworth Scale 2 — marked increase through ROM, limb easily moved). Subluxation: right shoulder 1 fingerbreadth inferior subluxation. Sling in use during upright activities.
Right Lower Extremity Moderate hemiparesis. Hip: 3/5. Knee: 3-/5. Ankle: 2+/5 (dorsiflexion weak — foot drop present). Tone: mildly increased (MAS 1+). AFO (ankle-foot orthosis) prescribed, not yet received.
Left Upper Extremity 5/5 — fully intact
Left Lower Extremity 5/5 — fully intact

Sensation

Right side: diminished light touch (upper and lower extremity), diminished proprioception (right hand and foot), intact pain/temperature.
Left side: intact all modalities.
Speech / Language (Aphasia Assessment):

Type: Expressive (Broca's) aphasia, moderate severity.

Comprehension: Relatively preserved — follows 2-step commands consistently, occasional difficulty with 3-step commands.

Expression: Limited to 2-3 word phrases; frequent word-finding difficulty; telegraphic speech ("want... water... please"); high frustration tolerance noted. Can nod/shake head for yes/no reliably.

Reading: Able to read single words and short phrases; difficulty with paragraphs.

Writing: Unable with dominant (right) hand due to hemiparesis. Left-hand writing: illegible — just beginning to practice.

Communication aids currently in use: Yes/No board (laminated); picture communication board (basic ADL needs, pain, emotions); husband/daughter assist with interpretation of gestures.

Pre-stroke language: Fully bilingual English/Japanese. Currently reverts to Japanese words/phrases when fatigued, frustrated, or emotional.

Family confirms she understood both languages equally pre-stroke.

Swallowing / Dysphagia

Modified Barium Swallow (MBS) at rehab (06/12/2025): Mild-moderate oropharyngeal dysphagia with delayed pharyngeal swallow trigger; reduced base of tongue retraction; trace penetration with thin liquids (PAS 3), no aspiration observed.

Current Diet Order: Mechanical soft solids / thin liquids (IDDSI Level 5 minced & moist; IDDSI Level 0 thin liquids). (Note: updated from initial thickened liquids at acute admission — thin liquids cleared at rehab discharge per SLP.)

Swallowing Precautions: Upright $\geq 90^\circ$ during meals and 30 minutes after; small bites/sips; alternate solids and liquids; check for oral pocketing (right cheek) after each bite; no straws (per SLP recommendation — sip from cup); cue for chin tuck if coughing.

Aspiration Risk: Moderate. Signs to monitor: wet/gurgling voice after swallowing, coughing during/after meals, throat clearing, pocketing food in right cheek, refusal to eat, recurrent fevers (aspiration pneumonia risk).

Suction machine: Available at bedside (portable) — for emergency use only (oral suctioning); husband trained on use at rehab.

Cognition

Alert and oriented x3 (person, place, time) — occasional confusion about date.

Attention: mildly impaired (difficulty sustaining attention for >10-15 minutes).

Memory: short-term recall impaired (2/3 objects at 5 minutes); long-term intact.

Judgment: mildly impaired — patient occasionally attempts to stand unassisted (impulsive, safety concern).

Executive function: impaired — difficulty with multi-step problem solving and sequencing.

Visual-spatial: right-sided neglect suspected — frequently fails to notice objects on right side of visual field; bumps right arm/leg into doorframes and furniture. Formal testing recommended.

Emotional / Behavioral

Emotional lability: present. Patient cries easily (triggered by frustration with speech, discussion of stroke, or seeing family members). Episodes are brief (1-2 minutes), not sustained. Patient indicates awareness of lability ("sorry... can't stop... crying").

Post-stroke depression: PHQ-2 positive (unable to complete full PHQ-9 due to aphasia — approximated score via clinician-administered modified version: ~12, moderate). Sertraline started 2 weeks ago.

Frustration: significant — primarily related to aphasia and functional dependence.

Motivation: good — actively engages in therapy, demonstrates effort.

No agitation, no aggression, no psychosis.

HOMEBOUND STATUS ASSESSMENT

Criterion Finding

Criterion 1 MET. Right hemiparesis with severe upper extremity involvement and moderate lower extremity involvement. Requires Hoyer lift for bed-to-wheelchair transfers. Unable to ambulate independently (requires rolling walker + max assist of 1 for standing pivot transfers only — not yet ambulatory). Right foot drop without AFO. Aphasia limits ability to communicate needs and navigate in community. Considerable and taxing effort to leave home — requires wheelchair, Hoyer lift or 2-person assist for vehicle transfer, and a companion for communication.

Criterion 2 MET. Absences limited to medical appointments only (neurology, psychiatry).

Homebound Status CONFIRMED

ADVANCE DIRECTIVES

Document Status

AHCD Yes — executed 2019. Husband David designated as primary agent; daughter Keiko as alternate.

POLST Yes — completed 06/02/2025 (during acute hospitalization). Full treatment (Section A); full treatment including intubation (Section B); long-term artificial nutrition: trial period, reassess in 90 days (Section D).

DNR No — Full Code

SERVICES ORDERED

Service Discipline Frequency Duration

Skilled Nursing RN 3x/week x 4 weeks, then 2x/week x 4 weeks 8 weeks

Physical Therapy PT 5x/week x 4 weeks, then 3x/week x 4 weeks 8 weeks

Occupational Therapy OT 5x/week x 4 weeks, then 3x/week x 4 weeks 8 weeks

Speech-Language Pathology SLP 5x/week x 4 weeks, then 3x/week x 4 weeks 8 weeks

Medical Social Work MSW 1x/week x 4 weeks, then 2x/month 8 weeks

Home Health Aide HHA Daily (AM — personal care, mobility assist) Duration of need

SKILLED NURSING FOCUS AREAS

Neurological Monitoring — Neuro checks each visit (LOC, orientation, pupil response, motor strength bilateral, speech quality, new deficits). NIH Stroke Scale (NIHSS) trend monitoring. Recognize signs of recurrent stroke.

Anticoagulation Management — Apixaban compliance; monitor for bleeding (critical given new A-fib anticoagulation); educate husband on signs of major bleeding, when to call 911.

Dysphagia Management — Assess swallowing safety each visit; monitor weight weekly (significant weight loss trajectory); calorie count x 3 days if intake remains poor; coordinate with SLP for diet advancement or regression; aspiration pneumonia surveillance (lung sounds, temperature, SpO2).

Medication Crushing Protocol — Verify which medications can be crushed (NOT Toprol XL); teach husband correct technique; assess for alternative formulations with physician for medications that cannot be crushed.

Skin Integrity — Braden Scale: 13 (moderate risk). Assess right shoulder sling fit (skin breakdown risk). Repositioning schedule q2h when in bed, q1h when in wheelchair. Pressure-relieving cushion for wheelchair. Inspect bony prominences each visit.

Urinary Management — Monitor neurogenic bladder symptoms; timed voiding schedule; I&O monitoring if ordered; UTI surveillance (previous UTI inpatient).

Depression/Emotional Lability Monitoring — Assess response to sertraline; coordinate with MSW/SLP for communication-based coping strategies.

Caregiver Training — Husband David is 79 with arthritis and uses a cane. Caregiver burden is a critical concern. Teach: Hoyer lift operation (both Keiko and David), transfer techniques, dysphagia precautions/meal preparation, medication crushing, suctioning, skin inspection, when to call agency vs. 911. Assess David's physical and emotional capacity at each visit.

PLAN OF CARE GOALS (60-DAY CERTIFICATION PERIOD)

Goal Target

- 1 No recurrent stroke or TIA events 08/17/2025
- 2 No aspiration pneumonia events 08/17/2025
- 3 Weight stabilized (no further loss; target ≥ 140 lbs) 08/01/2025
- 4 Right UE strength improved to $\geq 3/5$ at shoulder and elbow 08/17/2025
- 5 Right LE strength improved to $\geq 4/5$ at hip and knee 08/17/2025
- 6 Ambulate 50 feet with AFO, rolling walker, and contact guard assist 08/17/2025
- 7 Standing pivot transfer with moderate assist of 1 (eliminate Hoyer lift) 08/01/2025
- 8 Produce 4-5 word sentences with 60% intelligibility on functional topics 08/17/2025
- 9 Diet advanced to regular/modified texture per SLP reassessment 08/17/2025
- 10 Right neglect compensatory strategies demonstrated independently 08/17/2025
- 11 ADL participation: upper body dressing with setup only (left-hand dominant retraining) 08/17/2025
- 12 Husband demonstrates all caregiver tasks independently and safely 07/15/2025
- 13 PHQ-equivalent depression score improved to < 10 08/17/2025
- 14 Skin intact, no pressure injuries 08/17/2025
- 15 Zero falls 08/17/2025
- 16 Effective communication system established (AAC device or advanced communication board) 07/31/2025

PATIENT SIGNATURES — EVALUATION 3

Patient Signature (left hand): _____ Date: _____

(Patient signed with left hand — non-dominant; verified patient identity and understanding verbally and via communication board)

Print Name: Yuki Tanaka-Morrison

Husband / Health Care Agent Signature: _____ Date: _____

Print Name: David Morrison

Interpreter Used: ☐ Yes — Japanese telephone interpreter (Language Line) for complex medical discussion portions.

Clinician Signature: _____ Date: _____

Patient / Representative Signature

Date

RN Clinician Signature

Date / Time